

Complications of pregnancy & labor

Certain conditions classify the pregnancy as high risk. A high-risk pregnancy is closely monitored with more frequent prenatal appointments, fetal monitoring, and possibly additional ultrasounds. In labor, certain complications require immediate intervention.

Pregnancy complications

Miscarriage Early pregnancy loss is the most common complication during the first trimester and happens in 10 percent of known pregnancies.

Ectopic pregnancy occurs when a fertilized egg implants outside of the uterine cavity, most commonly in the Fallopian tube.

Causes Any woman can have an ectopic pregnancy, but the risk is increased if you've had a pelvic infection; became pregnant with a progesterone-releasing IUD in place, while taking the mini-pill, or as a result of fertility treatment; have endometriosis; have had abdominal surgery or a previous ectopic pregnancy.

Symptoms Most women who have an ectopic pregnancy notice pain and light bleeding at 6–8 weeks (2–4 weeks after a missed period). The pain is usually felt on one side of the lower abdomen and may be severe and persistent. If an ectopic pregnancy isn't recognized early and an embryo growing in the Fallopian tube ruptures the tube, you may feel sudden severe pain that spreads across the abdomen. Internal bleeding from a ruptured tube can also irritate the diaphragm, causing shoulder pain. If you have severe lower abdominal pain, call your doctor and go to the ER.

What might be done A ruptured ectopic pregnancy is life-threatening. Usually, an ectopic pregnancy is suspected before this stage. In this case, you'll have an ultrasound, usually through the vagina, which often diagnoses the problem; there is no pregnancy seen in the uterus; blood may be seen in the abdomen; and sometimes the ectopic pregnancy itself can be seen. You may also have blood tests to monitor the levels of hCG (pregnancy hormone); if levels of hCG plateau or rise slightly, this indicates an abnormal pregnancy. If an ectopic pregnancy hasn't been confirmed by these investigations, you may be offered a D&C to diagnose the location of the pregnancy.

If needed, surgery to remove the Fallopian tube is commonly performed by laparoscopy, a minimally invasive technique, or an incision at the bikini line. If diagnosed early, an ectopic pregnancy can be treated with methotrexate, which stops the pregnancy from developing. This is appropriate only if the hCG levels are low and the tube hasn't ruptured. The advantage is that surgery is avoided; however, the treatment doesn't always work, can be associated with significant pain, and close follow-up is vital.

Hyperemesis gravidarum Most women experience some nausea in pregnancy. Occasionally, vomiting may be severe. If you're unable to hold down food or drink for more than 24 hours, see your doctor.

What might be done Your urine may be checked for infection, and you may have an ultrasound if the pregnancy has not been confirmed yet. Your weight will be monitored, as weight loss of 10 percent of your body weight can increase the risk of complications. If you're very dehydrated, your doctor may advise a short hospitalization so you can receive intravenous fluids, and you may be given anti-nausea drugs and a vitamin supplement. Hyperemesis usually disappears by 13 weeks.

Anemia is a low level of hemoglobin, the oxygen-carrying component of red blood cells. Mild anemia is common because the extra fluid content of blood dilutes the number of